

Pfizer-BioNTech COVID-19 Vaccine

EUA 27034

**Response to CBER Information Request Received on
02 December 2020 Regarding (b) (6) Missing Information in the
Request for Emergency Use Authorization Submission**

December 2020

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1. INTRODUCTION

Reference is made to Request for Emergency Use Authorization (EUA 27034) submitted 20 November 2020 by Pfizer on behalf of BioNTech. Further reference is made to CBER's 02 December 2020 Information Request regarding (b) (6) missing information communicated by email from (b) (6) (CBER), to Elisa Harkins (Pfizer, Inc.).

CBER comments are presented in ***bold italics*** and Pfizer-BioNTech's responses follow in plain text.

2. CBER COMMENT

1. During the recent inspection of site number 1125 for the study C4591001, FDA investigators noted missing information such as toxicity grading, respiratory treatment, or hospitalization for COVID-19 illness visit in subject source documents which are supposed to be collected from subject eDiaries through the TrialMax app.

a. When the source documents are missing information such as toxicity grading, respiratory treatment, or hospitalization for a specific COVID -19 illness visit, describe your plans and procedures implemented: (i) to have such information entered by the clinical sites in the CRFs; (ii) for data monitoring; (iii) to initiate CAPAs. If data clarification forms (DCFs) were generated to request entry of such information in CRFs, please describe the rationale for such requests in the absence of missing information in source documents. Please include how many such occurrences you encountered in your study C4591001 at the US sites, if applicable.

Response

The TrialMax app does not capture toxicity grading, respiratory treatment, or hospitalization for COVID-19 illness. The eDiary question asks about presence of potential COVID-19 symptoms or diagnosis with a single Yes/No response which is designed to prompt contact with the investigator site.

The Site Monitors conduct source document review (SDR) and source data verification (SDV) on all COVID-19 Illness eCRFs contributing to the efficacy endpoint analysis. If source data were missing or discrepant with the eCRF entry, the Site Monitor raises a query to manage the discrepancy to resolution. In cases where the data was recorded or documented outside the study site (eg. external Emergency Room, hospital, or other clinic visit), a request for source from external location is initiated and that external source documentation added to the site's study documentation.

In addition to the Site Monitor SDR and SDV, the Data Managers and Clinical teams reviewed and cleaned eCRF data. During this process, in some cases where required forms (required COVID-19 Illness data outlined in section 8.29 of the CRF Completion Requirements) were either incomplete or discrepant with other entries, the Data Management or Clinical team raises queries directed to the site in the InForm database seeking clarification, and this may align before or after the Site Monitor performs SDR and SDV.

The Good Clinical Practice/ALCOACCEA compliance expectations set in the Initiation Visit, Investigator Meeting, and section 3.4 of the CRF Completion Requirements, that any eCRF entry must be supported by adequate and appropriate source documentation. In cases where the Site Monitor detects non-compliance or chronic errors or issues with data quality, additional monitoring review is conducted. If errors are more substantial, and potentially impact participant safety or reliability of data collected, then the Site Monitor follows escalation processes to share with study team leadership for additional evaluation and action.

There were 12 US sites at which Site Monitors performed increased SDR/SDV due to errors detected in sites' process or transcription. There were 55 US sites with quality issues escalated to the study team leadership for additional review, and CAPAs developed and managed through resolution.

b. The CRF Completion Requirements document collected from the site instructs the sites that adverse events can be reported on the CRF as deemed necessary by the clinical investigator even if such data are missing in subject eDiaries that are supposed to be collected through TrialMax app. Please clarify how you reconciled the missing data to be collected through TrialMax app for your study data submission in EUA 27034 with documentation.

Response

Study participants in the reactogenicity subset are asked to complete the reactogenicity eDiary in the evening for 7 days after each vaccination. For reactogenicity subset participants, reactogenicity data is not expected to be entered into the CRF. However, if an investigator discovers that a participant experienced one of the prompted symptoms but did not complete their eDiary, those data can be reported in the CRF.

The Site Monitors perform source document review (SDR) and source data verification (SDV) at each monitoring visit, following the study-specific rules and guidelines. In cases where an AE is reported in source, not documented in the eDiary, and not present in the CRF, the Site Monitor raises a query to the site's attention and manages to resolution.

Document Approval Record

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